



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : حنان زكى احمد عبدالخالق
المحافظة : القاهرة
الحى / المدينة : البساتين
رقم البطاقة : 26205070102624 رقم التليفون : 01143061709
العمر (تاريخ الميلاد) : 07-05-1962 المهنة : ربه منزل
العنوان : ..
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: حنان زكي احمد عبدالخالق National ID: 26205070102624	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



حنان
زكى احمد عبد الخالق
١٢ ح عبد الكريم من فرج يوسف
البيسكيتين - القاهرة

٢ ٦٢ ٠٥ ٠٧ ٠١ ٠٢ ٦ ٢٤

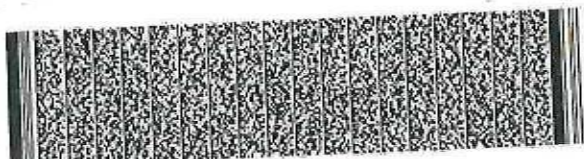
LL0249487

٢٠٢٥/١٠ ٢٦٢٠٥٠٧٠١٠٢٦٢٤

حاصله على دبلوم تجاره

أنثى
متزوجة
ابراهيم سمير احمد عمر
البطاقة سارية حتى ٢٠٢٢/١٠/٢٠





MDT FORM

02/23/2026

Name:	Hanan Zaky Ahmed Adbd Elkhalek	MRN:	5998
Age:	64 Y	ID:	
Menopausal Sta		Phone:	01151131618
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	POST OP

Clinical Data:

BX:
 ER: 8/8 PR: 8/8 HER2: +1 KI67: 20%
 R L

Sonomammography:

Mets Workup:

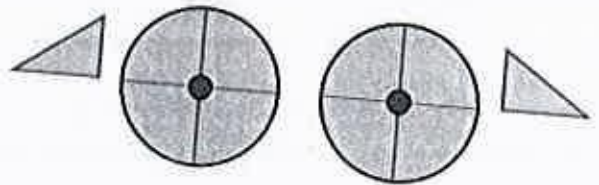
History:

Recommendations:

POST RT SM+SLNB
IDC G2
DEEP MARGIN FREE
NIPPLE AND AREOLA FREE
LN 0/7
T2N0

MDT Outcome & Plan

for adj ET AI 5 years + zometa /6m
In view of in-availability of Oncotype Dx : discuss benefit with patient and
consider adj TC 4 cycles versus No chemo



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

ent Name : الأستاذ / حنان زكي احمد	Request No. : 13009920
ent No. : 26205070102624	Request Date : 29/10/2025
No. : 3299/25	Verification Date : 05/11/2025
: Female	Financial Category : جوستاف روسي - مبانره
: 63 Y	Department : Out Patient
erreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Initial Data
 ght breast LOQ mass, BIRADS 4c
 Nature of specimen
 u cut needle biopsy collected on 28/10/2025.

Gross examination
 Fragmented grey white tissue core ranging in size from 0.2 up to 0.6 c., totally submitted .
 Technique:
 Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: ER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).
 Microscopic examination
 Histological type: Invasive duct carcinoma.
 Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
 DCIS: Focal , cribriform pattern, low grade .
 Lobular intra epithelial neoplasia : Absent
 Angioinvasion: Absent .
 Microcalcification :Absent
 Perineural invasion: Absent
 IPT (Positive control showed appropriate reactivity) :
 ER: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8)
 PR: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8)
 HER2: Faint incomplete membrane reaction in >10 % of tumor cells (1+)
 Ki 67: 20 %

Verified by :



Patient Name : الأستاذه / حنان زكي احمد
Patient No. : 26205070102624
Path No. : 3836/25
Sex : Female
Age : 63 Y
Referring Doc :

Request No. : 13003916
Request Date : 11/12/2025
Verification Date : 18/12/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis

1) Cervical polypectomy :

Cervical polyp with retention cysts.

2)D&C:

-Submucous leiomyoma with surface atrophic endometrium

-Cervical endo &ecto cervical epithelium with chronic cervicitis. .

Signature

Dr. Walaa Ibrahim Ghoneim

Dr.Heba Hassan Adly

Verified by :

Dr.Heba Hasan

Printing Date : 21/12/2025 15:01:58

Page 2 of 2

Printing By : .marwa mohamed youss

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt





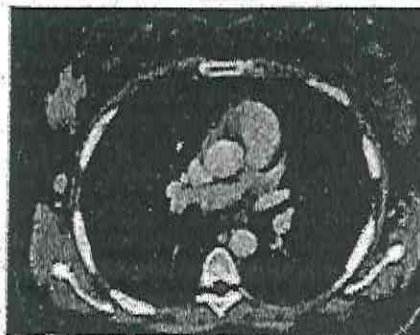
Patient Name: Hanan Zaky Ahmed Abd Al Khalek	Date : 16/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 3277/2025	Pat. ID: 132439

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:**Known case of breast cancer:****CT CHEST:**

- Right breast mass lesion is seen measures about 3 x 4.6 cm.
- Multiple bilateral axillary level I lymph nodes, all show preserved fatty hilae.
- Left upper lung lobe pneumatocele.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

**CT ABDOMEN AND PELVIS:**

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen and pancreas. Multiple right renal dense stones and right PUG largest stone measures about 2.5 x 2.4 cm reflecting moderate hydronephrosis.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

CT SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Opinion

- Right breast mass lesion as described.
- Multiple bilateral axillary level I lymph nodes.
- No Hepatic, pulmonary or osseous lesions could be seen.

Dr. Heba El Balshty, M

*Lecturer and Consultant of Diagnostic and Interventional Radiology
NLI-Menoufia Univ*

Patient's Name:

Hanan Zaki Ahmed Abdelkhalek

Date:

2026-08-31

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST:

TECHNIQUE: Multislice axial CT images of the chest with multiplanar reconstruction. IV contrast was used.

FINDINGS:

- Evidence of right mastectomy.
- Bilateral few scattered subcentimetric pulmonary nodules, largest one seen at right middle lobe measuring about 4mm
- Mosaic attenuation with bilateral areas of air trapping.
- Left upper lobe air filled cyst
- no consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly
- Small sliding hiatus hernia

Opinion:-

Bilateral few scattered subcentimetric pulmonary nodules for follow up

Dr.Nourhan Ewis

Dr.Nourhan Ewis

Patient's Name: Hannan Zaki

Date: 2026-09-24

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt

Pelvi-abdominal ultrasonographic examination, revealed:

- The liver is mildly enlarged measuring about 17 cm with bright homogeneous echo-pattern. No definite focal lesions or IHBDR by US criteria.
- Patent portal vein with average diameter showing normal flow direction.
- Gall bladder is averagely dilated with normal wall thickening. No stones or mud seen. Non-dilated CBD.
- Spleen is not enlarged measuring about 10 cm with homogenous echo pattern and no definite focal lesions.
- The midline structures are masked by gases.
- Colonic gaseous distension.
- Right renal upper polar two cortical cysts, largest about 3.5 X 4 cm, both kidneys are average in size, shape, echo-pattern and differentiation. No backpressure changes.
- No ascites at time of examination.
- No gross uterine or adnexal lesions.
- No pelvic fluid collection.

Conclusion:

- Bright (? Fatty) hepatomegaly, for clinical and lab correlation.
- Right renal upper polar two cortical cysts (Bosnick I).
- Otherwise, unremarkable study.

Best regards;

Dr. Mohammad Islam

Medical No. : 8971

Patient's Name: Hanan Zaki Ahmed Abdelkhalak

Date: 2026-09-24

Referred by: Prof. Dr. no doctor

Hospital: Goustave Roussey International Egypt



Left Digital soft tissue mammography with high resolution sonography

revealed:

Clinical summary: Status post right MRM routine follow up.

Findings:

Left breast.

- ❖ Scattered fibro-glandular densities. (ACR b)
- ❖ No speculations, masses , asymmetry, distortion or suspicious microcalcifications.
- ❖ scattered benign calcification
- ❖ **Ultrasound** still shows two well defined oval hypoechoic solid lesion seen at 12 o'clock measuring 20x6mm (it was 18x6mm) and retro areolar measuring 15x5mm (it was 15x5mm) no cystic lesions.
- ❖ Normal skin/nipple/areola complex.
- ❖ Non-specific axillary LNs.

Right mastectomy bed.

- ❖ Clear mastectomy bed with no masses or fluid collections.
- ❖ Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

●

Conclusion:-

- *Left breast (BIRADS 3): stationary course of benign looking lesions*
- *Clear right mastectomy bed and axilla.*

Best Regards,

DR. Marwa M.Hassan

Patient's Name:

Hanan Zaki Ahmed Abdelkhalek

Date:

2026-08-31

Referred by:

Prof.Dr. no doctor

Hospital:

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Date: 2026-09-24

Referred by: Prof. Dr. no doctor

Hospital: Goustave Roussey International Egypt



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DR. Marwa M.Hassan